FAX TRANSMITTAL

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Specialty: Letters
Sample Name: Rolandic Epilepsy

City, State,Dear Dr. Y:,I had the pleasure of seeing ABC today back in Neurology Clinic where he has been followed previously by Dr. Z. His last visit was in June 2006, and he carries a diagnosis of benign rolandic epilepsy. To review, his birth was unremarkable. He is a second child born to a G3, P1 to 2 female. He has had normal development, and is a bright child in 7th grade. He began having seizures, however, at 9 years of age. It is manifested typically as generalized tonic-clonic seizures upon awakening or falling into sleep. He also had smaller spells with more focal convulsion and facial twitching. His EEGs have shown a pattern consistent with benign rolandic epilepsy (central temporal sharp waves both of the right and left hemisphere). Most recent EEG in May 2006 shows the same abnormalities.,ABC initially was placed on Tegretol, but developed symptoms of toxicity (hallucinations) on this medication, he was switched to Trileptal. He has done very well taking 300 mg twice a day without any further seizures. His last event was the day of his last EEG when he was sleep deprived and was off medication. That was a convulsion lasting 5 minutes. He has done well otherwise. Parents deny that he has any problems with concentration. He has not had any behavior issues. He is an active child and participates in sports and some motocross activities. He has one older sibling and he lives with his parents. Father manages Turkey farm with foster farms. Mother is an 8th grade teacher.,Family history is positive for a 3rd cousin, who has seizures, but the specific seizure type is not known. There is no other relevant family history.,Review of systems is positive for right heel swelling and tenderness to palpation. This is perhaps due to sports injury. He has not sprained his ankle and does not have any specific acute injury around the time that this was noted. He does also have some discomfort in the knees and ankles in the general sense with activities. He has no rashes or any numbness, weakness or loss of skills. He has no respiratory or cardiovascular complaints. He has no nausea, vomiting, diarrhea or abdominal complaints.,Past medical history is otherwise unremarkable.,Other workup includes CT scan and MRI scan of the brain, which are both normal.,PHYSICAL EXAMINATION:,GENERAL: The patient is a well-nourished, well-hydrated male in no acute distress. VITAL SIGNS: His weight today is 80.6 pounds. Height is 58-1/4 inches. Blood pressure 113/66. Head circumference 36.3 cm. HEENT: Atraumatic, normocephalic. Oropharynx shows no lesions. NECK: Supple without adenopathy. CHEST: Clear auscultation.,CARDIOVASCULAR: Regular rate and rhythm. No murmurs. ABDOMEN: Benign without organomegaly. EXTREMITIES: No clubbing, cyanosis or edema. NEUROLOGIC: The patient is alert and oriented. His cognitive skills appear normal for his age. His speech is fluent and goal-directed. He follows instructions well. His cranial nerves reveal his pupils equal, round, and reactive to light. Extraocular movements are intact. Visual fields are full. Disks are sharp bilaterally. Face moves symmetrically with

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normal sensation. Palate elevates midline. Tongue protrudes midline. Hearing is intact bilaterally. Motor exam reveals normal strength and tone. Sensation intact to light touch and vibration. His gait is nonataxic with normal heel-toe and tandem. Finger-to-nose, finger-nose-finger, rapid altering movements are normal. Deep tendon reflexes are 2+ and symmetric.,IMPRESSION: ,This is an 11-year-old male with benign rolandic epilepsy, who is followed over the past 2 years in our clinic. Most recent electroencephalogram still shows abnormalities, but it has not been done since May 2006. The plan at this time is to repeat his electroencephalogram, follow his electroencephalogram annually until it reveres to normal. At that time, he will be tapered off of medication. I anticipate at some point in the near future, within about a year or so, he will actually be taken off medication. For now, I will continue on Trileptal 300 mg twice a day, which is a low starting dose for him. There is no indication that his dose needs to be increased. Family understands the plan. We will try to obtain an electroencephalogram in the near future in Modesto and followup is scheduled for 6 months. Parents will contact us after the electroencephalogram is done so they can get the results.,Thank you very much for allowing me to access ABC for further management.

Specialty: Nephrology
Sample Name: Renal Transplant - Cadaveric

HISTORY OF PRESENT ILLNESS: ,The patient is a 50-year-old African American female with past medical history significant for hypertension and endstage renal disease, on hemodialysis secondary to endstage renal disease, last hemodialysis was on June 22, 2007. The patient presents with no complaints for cadaveric renal transplant. After appropriate cross match and workup of HLA typing of both recipient and cadaveric kidneys, the patient was deemed appropriate for operative intervention and transplantation of kidney.,PREOPERATIVE DIAGNOSIS:, Endstage renal disease.,POSTOPERATIVE DIAGNOSIS: , Endstage renal disease.,PROCEDURE:, Cadaveric renal transplant to right pelvis.,ESTIMATED BLOOD LOSS: , 400 mL.,FLUIDS: ,One liter of normal saline and one liter of 5% of albumin.,ANESTHESIA: ,General endotracheal.,SPECIMEN: ,None.,DRAIN: , None.,COMPLICATIONS: , None.,The patient tolerated the procedure without any complication.,PROCEDURE IN DETAIL: ,The patient was brought to the operating room, prepped and draped in sterile fashion. After adequate anesthesia was achieved, a curvilinear incision was made in the right pelvic fossa approximately 9 cm in length extending from the 1.5 cm medial of the ASIS down to the suprapubic space. After this was taken down with a #10 blade, electrocautery was used to take down tissue down to the layer of the subcutaneous fat. Camper's and Scarpa's were dissected with electrocautery. Hemostasis was achieved throughout the tissue plains with electrocautery. The external oblique aponeurosis was identified with musculature and was entered with electrocautery. Then hemostats were entered in and dissection continued down with electrocautery down through the external internal obliques and the transversalis fascia. Additionally, the rectus sheath was entered in a linear fashion. After these planes were entered using electrocautery, the retroperitoneum was dissected free from the transversalis fascia using blunt dissection. After the peritoneum and peritoneal structures were moved medially and superiorly by blunt dissection, the dissection continued down bluntly throughout the tissue planes removing some alveolar tissue over the right iliac artery. Upon entering through the transversalis fascia, the epigastric vessels were identified and doubly ligated and tied with #0 silk ties. After the ligation of the epigastric vessels, the peritoneum was bluntly dissected and all peritoneal structures were bluntly dissected to a superior and medial plane. This was done without any complication and without entering the peritoneum grossly. The round ligament was identified and doubly ligated at this time with #0 silk ties as well. The dissection continued down now to layer of the alveolar tissue covering the right iliac artery. This alveolar tissue was cleared using blunt dissection as well as electrocautery. After the external iliac artery was identified, it was cleared circumferentially all the way around and noted to have good flow and had good arterial texture. The right iliac vein was then identified, and this was cleared again using electrocautery and blunt dissection. After the right iliac vein was

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identified and cleared off all the alveolar tissue, it was circumferentially cleared as well. An additional perforating branch was noted at the inferior pole of the right iliac vein. This was tied with a #0 silk tie and secured. Hemostasis was achieved at this time and the tie had adequate control. The dissection continued down and identified all other vital structures in this area. Careful preservation of all vital structures was carried out throughout the dissection. At this time, Satinsky clamp was placed over the right iliac vein. This was then opened using a #11 blade, approximately 1 cm in length. The heparinized saline was placed and irrigated throughout the inside of the vein, and the kidney was pulled into the abdominal field still covered in its protective socking with the superior pole marked. The renal vein was then elevated and identified in this area. A 5-0 double-ended Prolene stitch was used to secure the renal vein, both superiorly and inferiorly, and after appropriately being secured with 5-0 Prolene, these were tied down and secured. The renal vein was then anastomosed to the right iliac vein in a circumferential manner in a running fashion until secured at both superior and inferior poles. The dissection then continued down and the iliac artery was then anastomosed to the renal artery at this time using a similar method with 5-0 Prolene securing both superior and inferior poles. After such time the 5-0 Prolene was run around in a circumferential manner until secured in both superior and inferior poles once again. After this was done and the artery was secured, the Satinsky clamp was removed and a bulldog placed over. The flow was then opened on the arterial side and then opened on the venous side to allow for proper flow. The bulldog was then placed back on the renal vein and allowed for the hyperperfusion of the kidney. The kidney pinked up nicely and had a good appearance to it and had appearance of good blood flow. At this time, all Satinsky clamps were removed and all bulldog clamps were removed. The dissection then continued down to the layer of the bladder at which time the bladder was identified. Appropriate area on the dome the bladder was identified for entry. This was entered using electrocautery and approximately 1 cm length after appropriately sizing and incising of the ureter using the Metzenbaum scissors in a linear fashion. Before this was done, #0 chromic catgut stitches were placed and secured laterally and inferiorly on the dome of the bladder to elevate the area of the bladder and then the bladder was entered using the electrocautery approximately 1 cm in length. At this time, a renal stent was placed into the ureter and secured superiorly and the stent was then placed into the bladder and secured as well. Subsequently, the superior and inferior pole stitches with 5-0 Prolene were used to secure the ureter to the bladder. This was then run mucosa-to-mucosa in a circumferential manner until secured in both superior and inferior poles once again. Good flow was noted from the ureter at the time of operation. Additional Vicryl stitches were used to overlay the musculature in a seromuscular stitch over the dome of the bladder

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and over the ureter itself. At this time, an Ethibond stitch was used to make an additional seromuscular closure and rolling of the bladder musculature over the dome and over the anastomosis once again. This was inspected and noted for proper control. Irrigation of the bladder revealed that the bladder was appropriately filled and there were no flows and no defects. At this time, the anastomoses were all inspected, hemostasis was achieved and good closure of the anastomosis was noted at this time. The kidney was then placed back into its new position in the right pelvic fossa, and the area was once again inspected for hemostasis which was achieved. A 1-0 Prolene stitch was then used for mass closure of the external, internal, and transversalis fascias and musculature in a running fashion from superior to inferior. This was secured and knots were dumped. Subsequently, the area was then checked and inspected for hemostasis which was achieved with electrocautery, and the skin was closed with 4-0 running Monocryl. The patient tolerated procedure well without evidence of complication, transferred to the Dunn ICU where he was noted to be stable. Dr. A was present and scrubbed through the entire procedure.

Specialty: Neurology
Sample Name: Pituitary Adenomectomy

PREOPERATIVE DIAGNOSIS: , Large and invasive recurrent pituitary adenoma.,POSTOPERATIVE DIAGNOSIS:, Large and invasive recurrent pituitary adenoma.,OPERATION PERFORMED: , Endoscopic-assisted transsphenoidal exploration and radical excision of pituitary adenoma, endoscopic exposure of sphenoid sinus with removal of tissue from within the sinus, harvesting of dermal fascia abdominal fat graft, placement of abdominal fat graft into sella turcica, reconstruction of sellar floor using autologous nasal bone creating a cranioplasty of less than 5 cm, repair of nasal septal deviation, using the operating microscope and microdissection technique, and placement of lumbar subarachnoid catheter connected to reservoir for aspiration and infusion.,INDICATIONS FOR PROCEDURE: , This man has undergone one craniotomy and 2 previous transsphenoidal resections of his tumor, which is known to be an invasive pituitary adenoma. He did not return for followup or radiotherapy as instructed, and the tumor has regrown. For this reason, he is admitted for transsphenoidal reoperation with an attempt to remove as much tumor as possible. The high-risk nature of the procedure and the fact that postoperative radiation is mandatory was made clear to him. Many risks including CSF leak and blindness were discussed in detail. After clear understanding of all the same, he elected to proceed ahead with surgery.,PROCEDURE: ,The patient was placed on the operating table, and after adequate induction of general anesthesia, he was placed in the left lateral decubitus position. Care was taken to pad all pressure points appropriately. The back was prepped and draped in usual sterile manner.,A 14-gauge Tuohy needle was introduced into the lumbar subarachnoid space. Clear and colorless CSF issued forth. A catheter was inserted to a distance of 40 cm, and the needle was removed. The catheter was then connected to a closed drainage system for aspiration and infusion.,This no-touch technique is now a standard of care for treatment of patients with large invasive adenomas. Via injections through the lumbar drain, one increases intracranial pressure and produces gentle migration of the tumor. This improves outcome and reduces complications by atraumatically dissecting the tumor away from the optic apparatus.,The patient was then placed supine, and the 3-point headrest was affixed. He was placed in the semi-sitting position with the head turned to the right and a roll placed under the left shoulder. Care was taken to pad all pressure points appropriately. The fluoroscope C-arm unit was then positioned so as to afford an excellent view of the sella and sphenoid sinus in the lateral projection. The metallic arm was then connected to the table for the use of the endoscope. The oropharynx, nasopharynx, and abdominal areas were then prepped and draped in the usual sterile manner.,A transverse incision was made in the abdominal region, and several large pieces of fat were harvested for later use. Hemostasis was obtained. The wound was carefully closed in layers.,I then advanced a 0-degree endoscope up the left nostril. The middle turbinate was identified

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and reflected laterally exposing the sphenoid sinus ostium. Needle Bovie electrocautery was used to clear mucosa away from the ostium. The perpendicular plate of the ethmoid had already been separated from the sphenoid. I entered into the sphenoid.,There was a tremendous amount of dense fibrous scar tissue present, and I slowly and carefully worked through all this. I identified a previous sellar opening and widely opened the bone, which had largely regrown out to the cavernous sinus laterally on the left, which was very well exposed, and the cavernous sinus on the right, which I exposed the very medial portion of. The opening was wide until I had the horizontal portion of the floor to the tuberculum sella present.,The operating microscope was then utilized. Working under magnification, I used hypophysectomy placed in the nostril.,The dura was then carefully opened in the midline, and I immediately encountered tissue consistent with pituitary adenoma. A frozen section was obtained, which confirmed this diagnosis without malignant features.,Slowly and meticulously, I worked to remove the tumor. I used the suction apparatus as well as the bipolar coagulating forceps and ring and cup curette to begin to dissect tumor free. The tumor was moderately vascular and very fibrotic.,Slowly and carefully, I systematically entered the sellar contents until I could see the cavernous sinus wall on the left and on the right. There appeared to be cavernous sinus invasion on the left. It was consistent with what we saw on the MRI imaging.,The portion working into the suprasellar cistern was slowly dissected down by injecting saline into the lumbar subarachnoid catheter. A large amount of this was removed. There was a CSF leak, as the tumor was removed for the upper surface of it was very adherent to the arachnoid and could not be separated free.,Under high magnification, I actually worked up into this cavity and performed a very radical excision of tumor. While there may be a small amount of tumor remaining, it appeared that a radical excision had been created with decompression of the optic apparatus. In fact, I reinserted the endoscope and could see the optic chiasm well.,I reasoned that I had therefore achieved the goal with that is of a radical excision and decompression. Attention was therefore turned to closure.,The wound was copiously irrigated with Bacitracin solution, and meticulous hemostasis was obtained. I asked Anesthesiology to perform a Valsalva maneuver, and there was no evidence of bleeding.,Attention was turned to closure and reconstruction. I placed a very large piece of fat in the sella to seal the leak and verified that there was no fat in the suprasellar cistern by using fluoroscopy and looking at the pattern of the air. Using a polypropylene insert, I reconstructed the sellar floor with this implant making a nice tight sling and creating a cranioplasty of less than 5 cm.,DuraSeal was placed over this, and the sphenoid sinus was carefully packed with fat and DuraSeal.,I inspected the nasal passages and restored the septum precisely to the midline repairing a previous septal deviation. The middle turbinates

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were then restored to their anatomic position. There was no significant intranasal bleeding, and for this reason, an open nasal packing was required. Sterile dressings were applied, and the operation was terminated.,The patient tolerated the procedure well and left to the recovery room in excellent condition. The sponge and needle counts were reported as correct, and there were no intraoperative complications.,Specimens were sent to Pathology consisting of tumor.

LABORATORY REPORT

Packet: 007

Analyte	Value	Units	Flag
Glucose	142	mg/dL	High
Hemoglobin	11.8	g/dL	Low
WBC	12.4	10*3/uL	High
Sodium	136	mmol/L	
Potassium	4.2	mmol/L	
Creatinine	1.4	mg/dL	High
BUN	24	mg/dL	High
Platelets	228	10*3/uL	
Hemoglobin A1c	7.6	%	High
TSH	2.1	mIU/L	